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IN THE
Supreme Court of the United States
OCTOBER TERM, 1997

CEDAR RAPIDS COMMUNITY SCHOOL DISTRICT,
Petitioner,

v.

GARRET F., A MINOR BY HIS MOTHER AND NEXT FRIEND, CHARLENE F.,
Respondent.

On Writ of Certiorari to the United States Court of Appeals for the Eighth Circuit

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REPLY BRIEF FOR THE PETITIONER

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REPLY BRIEF FOR THE PETITIONER

This case presents the question whether the Court should steer away from the plain language of the statute, its own decisions, and over a decade of nearly unanimous lower court rulings to hold that school districts have an obligation to provide continuous one-on-one nursing services, on the pretense that such services are not really "medical" because they are not personally administered by a physician. The "medical services" exclusion in the IDEA must be given its plain, everyday, common-sense meaning. As a result, the lower court decisions in this case must be reversed.

First, there is no question that Garret needs continuous one-on-one nursing services provided by a nursing professional, and that such nursing services would result in substantial additional costs to the District. At the same time, there is no basis to conclude that if the District prevails in this case, Respondent would be relegated to homebound instruction. For ten years, Respondent has attended the

Cedar Rapids public schools, even though the intensive nursing services that he requires have *not* been provided by the District.

Second, because the statutory language is paramount, the District will refocus on the real issue in this case: Does the term "medical services" in the IDEA include medical services as that term is commonly understood? In a number of other contexts, Congress, federal agencies, and the courts have defined "medical" assistance, care or services as embracing many services that are not personally administered by a physician.

Third, the District will show that a series of developments since the IDEA was enacted in 1975, including administrative regulations and interpretations, this Court's *Tatro* decision, and lower court decisions that have been implicitly ratified by Congress, strongly support a common-sense, practical view of the term "medical." Additionally, because the IDEA was enacted under the Spending Clause, not under the Fourteenth Amendment, it is a contract with the States, and should not be deemed to impose requirements on school districts unless the requirements are stated "unambiguously" in the legislation.

Fourth, the District will demonstrate that balancing is at the heart of the IDEA, and that a balancing test to determine whether a service is "medical" is both logical and feasible. Unfortunately, the administrative law judge did not perform a true balancing in this case, relying instead on his erroneous view of the law as limiting "medical services" to those directly provided by a physician.

THE FACTS FULLY SUPPORT THE DISTRICT'S ARGUMENTS.

A. Garret Requires Continuous, One-on-One Nursing Services.

The factual record shows, without question, that Respondent requires continuous, one-on-one nursing services that would impose considerable additional costs on the District if it lost this case. Respondent's own attending

physician, Dr. Reinertson, testified that a trained individual must be within "earshot" of Garret at all times. TR 628, 630. Dr. Reinertson also signed a certification that Garret requires a one-on-one nurse to care for his needs. TR 55-57.

Merely because the LPN who currently accompanies Garret at all times is not constantly performing a procedure upon Garret does not mean that his care is not "continuous." All of the witnesses agreed that Garret needed a personal attendant with him at all times, and that the school nurse, who is responsible for several hundred students in multiple buildings (there are six nurses serving the 33 school buildings in the District) would not be able to meet his needs. TR 244, 317-320.

Garret needs constant monitoring and assessment to determine if he is suffering respiratory distress and to ensure that his ventilator is properly functioning. TR 44-46, 408-409, 427. Because Garret is paralyzed, someone must attend to these needs for him. Garret also has a history of episodes of autonomic hyperreflexia, a life-threatening abnormal response of the nervous system. TR 52, 77-78, 81-82, 192, 200. At the time of hearing, Garret's mother testified that Garret had one or two such episodes within the prior year, albeit while at home, *and that he had experienced such episodes while at school prior to then.* TR 81-82. Dr. Reinertson, Garret's pediatrician, also testified that Garret was susceptible to further episodes and thus required "immediate care." TR 77-78.

B. There Would Be Substantial Additional Costs if the District Had to Provide Garret's Continuous, One-on-One Nursing Services.

Respondent also alleges that the District could provide Garret's continuous one-on-one nursing services with no additional outlay of funds because his educational associate in the 1994-95 school year also happened to be a licensed practical nurse. However, Garret's educational

associate for that year was not hired for and did not provide for Garret's medical needs. She helped him maneuver his wheelchair and manipulate his educational materials, but did not participate in his nursing care. TR 238, 414, 472, 521. Her annual compensation of approximately $9,500 was far below the annual compensation for nurses. EX 128 at 463; EX 137 at 520-521. The record demonstrates, and the ALJ found, that the educational associate was covered by a collective bargaining contract which provides a salary and benefits much lower than what is paid to employees hired to provide nursing services. J.A. 17; EX 128 at 463; EX 135 at 483-523; Pet. App. 34a-36a. The ALJ found that the increased costs for nursing services would be between $19,082.57 and $30,262.42. Pet. App. 36a. In any event, the current educational associate is not an LPN.

Moreover, there can be no dispute that the Iowa Board of Nursing requires a licensed nurse (either an LPN or an RN) to attend to Garret continuously. At the time of the hearing, the District had received a declaratory ruling from the Iowa Board of Nursing that Garret's care at school could not be delegated to a nonlicensed trained individual. EX 125 at 447-461. The Board of Nursing also has a regulation that school nurses must be registered nurses, and that licensed practical nurses may not be assigned to perform school nursing services unless there is a registered nurse in the "proximate area," defined as the same building. 655 Iowa Admin. Code §§ 6.1 & 6.3(4). Later, a second declaratory ruling was received from the Board of Nursing, indicating that while Garret's care could not be delegated to nonlicensed personnel, an LPN could perform the services so long as an RN determined the services could be safely performed and retained ultimate responsibility for the care. *Letter from Lois Churchill* (June 13, 1997) (copy lodged with the Court by the United States). Thus, the Board of Nursing has reiterated that a licensed nurse must care for Garret at all times when he is at school.

C. Garret Will Not Be Homebound.

Respondent and his amici imply, generally without directly saying so, that if the District does not pay for Garret's continuous one-on-one nursing, he could be relegated to homebound instruction with a teacher for just a few hours a week, isolated from nondisabled peers. The implication is untrue. For the past ten years, Garret has never been excluded from school, and he has been provided a wide array of services at no expense to enable him to attend school in regular classes. Pet. App. 19a, TR 106-109, 217, 516, 517. These services include a one-on-one associate, thousands of dollars worth of special software, special equipment to enable him to manipulate his computer and a special lift bus to accommodate his wheelchair (which is larger and heavier than usual because of the ventilator). TR 207-208, 515-517.

Also, there are nondistrict resources available to pay for Garret's nursing services: a combination of private medical insurance and a settlement trust fund expressly established to cover Garret's needs. TR 89-93. The question is not whether Garret may attend school, but rather who pays for the nursing services which he requires both at home and school, while he is at school.

ARGUMENT

I. THE TERM "MEDICAL SERVICES" AS USED IN THE IDEA MUST HAVE A PRACTICAL, COMMONSENSE MEANING.

Any consideration of the IDEA's medical services exclusion must begin with the statute. The importance of the statutory language is paramount. *Connecticut Nat'l Bank v. Germain*, 503 U.S. 249, 253-254 (1992). "Medical services" must be given its "ordinary, contemporary, common meaning." *See Walters v. Metropolitan Educ. Enters., Inc.*, 117 S. Ct. 660, 664 (1997). Congress and several courts have repeatedly defined "medical" as including services beyond those actually administered by a physician, including nursing services. This traditional defi-

nition of "medical" reflects practical realities and common sense.

As the Seventh Circuit recently said in analyzing the medical services exclusion of the IDEA, a bright-line physician/nonphysician distinction is "unrealistic." *Morton Community Unit Sch. Dist. No. 709 v. J.M.*, 1998 U.S. App. LEXIS 17079 at *7 (7th Cir. July 27, 1998). Most medical services, "as a matter of economy and specialization, are provided by nurses, nurses' aides, paramedics, medical technicians, medical students, the patients themselves, and members of the patient's families, rather than by physicians." *Id.* Respondent and the United States' definition of "medical services" is not "contemporary"; it is archaic.

Respondent attempts to cite several federal statutes to support an unduly narrow definition of "medical" as "limited to services directly provided by a physician." But unlike the authorities cited by the District, such as the well-established definition of "medical care" in the Internal Revenue Code, 26 U.S.C. § 213(d)(1), none of these statutes cited by Respondent actually *defines* the term "medical." Moreover, far from supporting Respondent, many of the statutes cited by Respondent bolster the District's position that "medical" encompasses services performed by nonphysicians.

For example, 38 U.S.C. § 101(28), relating to veterans' benefits, does not actually define "medical services" but strongly implies that such services can be performed "under the general direction of" physicians. Moreover, another provision, 38 U.S.C. § 1701(6), *expressly* defines "medical services" to include rehabilitative services, surgical services, dental services, appliances, optometric and podiatric services, and preventive health-care services.

Likewise, 42 U.S.C. § 1397cc(c)(1)(B) suggests there is a subcategory of "medical services" provided by "physicians," and correspondingly there are medical services not provided by physicians. Additionally, while 5 U.S.C. § 8103(a) (also cited by Respondent) does not define

"medical services," the implementing regulations do, and the definition supports the District. *See* 20 C.F.R. § 10.400(e) (the term "medical services" includes "services and supplies provided by *or under the supervision of* physicians (M.D. and D.O.), surgeons, podiatrists, dentists, clinical psychologists, optometrists, and chiropractors. . . .") (emphasis added).

In summary, when Congress or a federal agency has defined the term "medical" in connection with "assistance," "care" or "services," none of the definitions has been limited to services directly administered by a physician.

II. THE LEGISLATIVE HISTORY OF THE IDEA, WHILE SPARSE, SHOWS THAT CONGRESS INTENDED A MEANINGFUL "MEDICAL SERVICES" EXCLUSION.

The legislative history of the IDEA as enacted in 1975 shows that Congress made a deliberate choice to exclude "medical services" (other than for diagnosis and evaluation) from the services that school districts had to provide. This was no mere accident.

As originally introduced in the 94th Congress, the precursors to the IDEA, S. 6 and H.R. 7217, defined "related services" to include "medical" services. The House and Senate Committees then conducted hearings in the spring of 1975, during which there was extensive discussion of the costs of special education. Before they were reported to the floor, both S. 6 and H.R. 7217 were modified to exclude medical services generally. *See* H.R. Rep. No. 332, 94th Cong., 1st Sess., June 26, 1975, at 42-43 (related services includes medical services, "except that such medical services shall be for diagnostic and evaluation purposes only"); S. Rep. No. 168, 94th Cong., 1st Sess., June 2, 1975, at 62 (related services does not include medical services). Ultimately, the Conference Committee approved the House language. S. Conf. Rep. No. 455, 94th Cong., 1st Sess., Nov. 14, 1975, at 30. Nothing in this legislative history supports or even suggests a

cramped interpretation of the term "medical services" to mean only services performed by a physician.

III. DEVELOPMENTS SINCE THE IDEA WAS ENACTED STRONGLY SUPPORT THE DISTRICT'S POSITION ON EXCLUDED MEDICAL SERVICES.

Since 1975, a series of developments have occurred which confirm that the "medical services" exclusion should not be limited artificially and exclusively to services which must be personally administered by a physician. These developments include: (1) regulations and administrative interpretations of the IDEA; (2) legislative amendments; (3) the Supreme Court's decisions in *Rowley* and *Tatro*; and (4) a succession of lower court rulings, including decisions by six separate United States Courts of Appeals, that have been implicitly ratified by Congress.

A. The Department of Education's Regulations and Interpretations Do Not Favor a Bright-Line Rule.

Respondent and the United States argue that deference should be given to the regulations of the Secretary of Education. However, the question is, "deference to what?" As previously discussed, the regulations have never defined *excluded* medical services. Nor has the Department prior to this case ever adopted a bright-line interpretation of the medical services exclusion in the context of continuous one-on-one nursing. *See generally J.M.*, 1998 U.S. App. LEXIS 17079 at *8 (characterizing the government's statements in this case about its own interpretations of the medical services exclusion as "inaccurate").

As promulgated in 1977, the regulations contained (and still contain to this day) a narrow definition of *included* medical services as "services provided by a licensed physician to determine a child's medically related [disability] that results in the child's need for special education and related services." 42 Fed. Reg. 42479 (1977). Given the concerns about the impending costs of complying with the Act, it is logical to conclude that the Department wished

to limit strictly the diagnostic and evaluative medical services that would be *covered*. The Department did not need to provide a definition of *excluded* medical services because "medical services" already had a common meaning. Had the Department wished to attach a restrictive meaning of *excluded* medical services, it could have simply adopted a regulation to that effect, just as it did for included diagnostic/evaluative medical services.

In January 1981, the Secretary of Education issued a notice of interpretation of these regulations which supports this view. 46 Fed. Reg. 4912 (1981). In that notice, the Secretary interpreted the IDEA and its regulations as requiring the provision of clean intermittent catheterization ("CIC") to disabled students during the school day. However, the result is less significant than the Secretary's reasoning.

Since CIC is not required to be provided by physicians, the Secretary could have made the required "interpretation" very quickly if she believed that the existing regulations established a "bright-line" rule limiting excluded medical services to physician-provided services. But the Secretary did not follow that approach, and in fact, did not even mention the included medical services regulation. Rather, she made a far lengthier and more balanced argument which emphasized that CIC was "a relatively simple procedure" that could be performed by a school nurse or another responsible person, "none of whom need to be licensed to perform that service." *Id.* The Secretary also said at the same time that school districts did not need to provide "routine medical services, such as laboratory analysis," even though such services are generally not administered by physicians. A bright-line interpretation of excluded medical services was clearly not contemplated in January of 1981.

Even more importantly, in May 1981, the Department indefinitely postponed its CIC interpretation, stating that the existing regulations were "ambiguous" concerning the differentiation between related services and medical serv-

ices. 46 Fed. Reg. 25614 (1981). It is inconceivable that the Department would have withdrawn the CIC interpretation because the regulations were "ambiguous," if it believed the regulations established a bright-line physician/nonphysician rule for excluded medical services. Such a bright-line rule would clearly have required school districts to provide CIC.

The Secretary's efforts to revise the regulations in 1982 do not alter this analysis. At most, the Secretary stated at that time that there were "problems . . . . determining whether certain health-related services are to be provided." 47 Fed. Reg. 33837 (1982). The Secretary did not say there had been or should be a "bright-line" rule governing *excluded* "medical services." While the Secretary's attempt to narrow sharply the definition of "related services" ran into congressional opposition and was later withdrawn, there is no indication that anyone believed continuous one-on-one nursing services would be a covered service or that only physician services would be excluded.

Yet again as recently as 1996, the Department could not "express a view as to whether one-on-one nursing services are a required related service." Pet. App. 67a. The United States argues that this recent interpretation from the Department merely "acknowledged that courts have reached differing conclusions" and that the letter "reaffirmed the Secretary's longstanding interpretation of IDEA that services provided by a nurse are not excluded medical services." However, the 1996 interpretation at no time mentions a longstanding bright-line distinction, nor does it ever say that services provided by a nurse are not excluded medical services. Rather,

> the determination as to whether [one-on-one nursing services] are a required related service for an individual disabled student must be made on a case-by-case basis. [Pet. App. 67a]

Again in 1996, the Department commented:

> If the services are not for diagnostic and evaluative purposes and could be provided by another qualified

> professional, they *may* be "school health services." The determination as to whether a particular related services is a "school health service" or a "medical service," not for diagnostic and evaluation purposes must be made on a case-by-case basis. [Letter to Anonymous, 25 Ind. Disab. Educ. L. Rep. 531, 532 (Nov. 13, 1996)] (emphasis added).

The Department could have answered these questions simply by saying that if necessary services need not be provided by a physician, they are "related services." It did *not* do so, thereby demonstrating the fallacy of the argument that the Department has consistently and clearly applied a physician/nonphysician distinction to defining "medical services." [1]

It is well-settled that inconsistent agency interpretations are entitled to considerably less deference. *INS v. Cardoza-Fonseca*, 480 U.S. 421, 446 n.30 (1987); *General Elec. Co. v. Gilbert*, 429 U.S. 125, 142-43 (1976). Moreover, an agency interpretation that conflicts with the plain language of the statute is entitled to no deference at all. *Public Employees Retirement Sys. of Ohio v. Betts*, 492 U.S. 158, 171 (1989); *Bowsher v. Merck & Co., Inc.*, 460 U.S. 824, 837 (1983).

B. Later Legislative Amendments to the IDEA Confirm That "Medical" Must Have a Common-Sense Meaning.

A 1986 amendment to the IDEA inserting the term "medical" into another section of the Act also supports

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[1] In its brief, the United States makes much of some policy letters that the Department issued in the late 1970's relating to the provision of psychotherapy services. Since "psychological services" and "counseling services" are expressly required to be provided by school districts under the IDEA, the Department confronted a different question than is presented here. The Department had to decide whether non-physician psychotherapy was a "psychological" service or a "medical" service, and in several letters, it concluded that it was a psychological service if not provided by a psychiatrist. These letters do not resolve the issue presented in this case, and indeed they were not cited in the Department's 1996 letters on continuous nursing services.

the District's position. In 1986, the following provision was added:

> This chapter shall not be construed to permit a State to reduce *medical* and other assistance available or to alter eligibility under titles V and XIX of the Social Security Act *with respect to the provision of a free appropriate public education* for children with disabilities within the State. [20 U.S.C. § 1413(e)] (emphasis added)

This supports the District's contention that "medical" means more than just physician services in the related services exclusion. Otherwise, no medical services would be listed in the amendment because they would not be deemed "with respect to the provision of a free appropriate public education," thus rendering the 1986 statutory provision meaningless.

Psychological, physical therapy and occupational therapy are also commonly viewed as "medical services"; thus these services needed to be specifically listed as related services in order to avoid falling under the express "medical service" exclusion of 20 U.S.C. § 1401(a)(17). However, it is noteworthy that Congress did not identify private-duty nursing as a related service, presumably then believing that it would be an excluded medical service.

C. This Court's Decisions Do Not Favor a Bright-Line Rule.

The question whether a school district was required to provide CIC under the IDEA reached this Court in *Irving Independent Sch. Dist. v. Tatro*, 468 U.S. 883 (1984). Some language in *Tatro* can be read as favoring a bright-line rule. But those statements were not necessary to the Court's decision, and they go against much of the Court's reasoning. For the most part, this Court emphasized in *Tatro* that CIC was not "beyond the range of [schools'] competence," was "difficult to distinguish from" services traditionally provided by school nurses to nondisabled children, and was a service of a kind "routinely provided

to the nonhandicapped." 468 U.S. at 892-94. These were the critical factors enunciated by the Court in determining that the Irving School District had to provide CIC to Amber Tatro. This balanced, nuanced approach was consistent with the Court's earlier decision in *Board of Educ. of Hendrick Hudson Central Sch. Dist. v. Rowley*, 458 U.S. 176 (1982).

D. Lower Court Decisions Do Not Favor a Bright-Line Rule.

The lower courts have overwhelmingly read *Tatro* as adopting a balanced approach to the "medical services" exclusion. If *Tatro* endorsed a bright-line physician/non-physician rule, it is difficult to explain why lower courts, almost without exception, have read *Tatro* otherwise. Six Circuit Courts of Appeals, as well as several lower courts, have held that the "medical services" exclusion is not limited to services directly provided by physicians.[2] The prevailing view, as the Fourth Circuit stated, is that "such an arbitrary classification ignores reality." *Tice*, 908 F.2d at 1209 n.13. Indeed, the Seventh Circuit recently used the terms "arbitrary" and "extreme" to describe the attempt to confine the term "medical services" to "services rendered by a licensed physician." *J.M.*, 1998 U.S. App. LEXIS 17079 at *13.[3]

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[2] *See Detsel v. Board of Educ. of Auburn Enlarged City Sch. Dist.*, 820 F.2d 587, 588 (2nd Cir. 1987) (per curiam), *aff'g* 637 F. Supp. 1022 (N.D. N.Y. 1986), *cert. denied*, 484 U.S. 891 (1987); *Fulginiti v. Roxbury Township Pub. Sch.*, 921 F. Supp. 1320 (D.N.J. 1996), *aff'd without published opinion*, 116 F.3d 468 (3rd Cir. 1997); *Tice v. Botetourt County Sch. Bd.*, 908 F.2d 1200, 1209 n.13 (4th Cir. 1990); *Neely v. Rutherford County Sch.*, 68 F.3d 965, 969-973 (6th Cir. 1995), *cert. denied*, 517 U.S. 1134 (1996); *Morton Community Unit Sch. Dist. No. 709 v. J.M.*, 1998 U.S. App. LEXIS 17079 at *6-8, *13 (7th Cir., July 27, 1998); *Clovis Unified Sch. Dist. v. California Office of Admin. Hearings*, 903 F.2d 635, 642-44 (9th Cir. 1990) (per curiam); *Bevin H. v. Wright*, 666 F. Supp. 71, 75 (W.D. Pa. 1987); *Granite Sch. Dist. v. Shannon M.*, 787 F. Supp. 1020, 1030 (D. Utah 1992); *Ellison v. Board of Educ. of Three Villages Central Sch. Dist.*, 597 N.Y.S.2d 483 (App. Div. 1993).

[3] The Seventh Circuit ultimately ruled for the disabled student based on "the principle of judicial deference to the factual, judg-

Except for the Eighth Circuit's opinion in this case, only three reported decisions have adopted the bright-line distinction since *Tatro* was decided fourteen years ago. The reasoning of all of those district court decisions has been disapproved by their respective appellate courts.[4]

The significance of this body of case law is that Congress has made no attempt to overturn these decisions legislatively, even as recently as June of 1997, when it amended the IDEA by adding "orientation and mobility services" to the list of required related services. Pub. L. 105-17, 111 Stat. 45. This is strong evidence that Congress agreed with the courts. Congress could not have been unaware of these decisions, particularly in light of the large body of literature that discussed them. While the United States argues that Congress "ratified" a bright-line rule it is far more logical to argue that Congress ratified the prevailing balancing approach.

IV. BECAUSE IT IS CLEARLY A SPENDING CLAUSE ENACTMENT, THE IDEA'S OBLIGATIONS ON SCHOOL DISTRICTS MUST BE NARROWLY CONSTRUED.

Respondent and the United States argue that the IDEA was enacted pursuant to Section 5 of the Fourteenth

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mental, or discretionary decisions of the front-line administrative decision-makers." *Id.* at *14. But such deference would be improper here because, as the District has already extensively argued, the ALJ fundamentally made a *legal* determination, *i.e.*, that the "medical services" exclusion is controlled by a bright-line physician/non-physician distinction. That legal determination was erroneous.

[4] *See Skelly v. Brookfield LaGrange Park Sch. Dist. 95*, 968 F. Supp. 385 (N.D. Ill. 1997); *Morton Community Unit Sch. Dist. No. 709 v. J.M.*, 986 F. Supp. 1112 (C.D. Ill. 1997); and *Macomb Intermediate County Sch. Dist. v. Joshua S.*, 715 F. Supp. 824 (E.D. Mich. 1989). Respondent cites *Tanya v. Cincinnati Bd. of Educ.*, 651 N.E.2d 1373 (Ohio App. 1995), as purportedly favoring a bright-line rule. But the critical points in that decision were that one-on-one nursing services were not required and that the necessary accommodation was "not unduly burdensome." *Id.* at 1376-77. The court expressly declined to reach the question whether full-time nursing constitutes "medical services" or "related services." *Id.* at 1377 n.1.

Amendment and, therefore, the usual rule of construction for Spending Clause enactments does not apply. However, *Rowley* squarely holds that the IDEA must be treated for statutory interpretation purposes as a Spending Clause enactment. 458 U.S. at 204 n.26.

Moreover, this argument ignores the position taken by the federal government at the very outset that the IDEA was enacted pursuant to the Spending Clause only. In 1976, when regulations implementing the IDEA were first proposed, the Department of Health, Education and Welfare (predecessor to the Department of Education) acknowledged that the IDEA was not a direct mandate but merely imposed a series of conditions on the receipt of funding. The Department said, "[A] State which elects not to receive a grant under Part B [of the IDEA] *is not subject to its provisions.*" 41 Fed. Reg. 56967 (1976) (emphasis added).

V. USING A MULTI-FACTOR TEST FOR "MEDICAL SERVICES" IS CONSISTENT WITH DETERMINATIONS MADE IN OTHER CONTEXTS UNDER THE IDEA.

A. Balancing Pervades the IDEA.

The District has proposed, consistent with numerous lower court decisions, to use a multi-factor test to determine if services are excluded "medical services," including examining the nature and extent of the services and the burden on the district. Respondent and his amici have argued this is an onerous test that will lead to inconsistent provision of services across the country and unnecessary litigation. However, the IDEA is premised upon the mandate that decisions are to be based on individualized determinations on a case-by-case basis, and many multi-factored and balancing tests are utilized under the IDEA.

The nature and extent of required services are examined in making determinations for Medicaid assistance and for private insurance coverage. These determinations go well beyond merely identifying whether the service must be performed by physicians. Medicaid, which is expressly defined as "medical assistance," may include "pri-

vate duty nursing." 42 U.S.C. § 1396d(r), incorporating services in § 1396d(a). In this case, Garret's private insurer determined that his needed one-to-one attendant services qualified as "medically necessary services," thus entitling him to payment for a nurse to accompany him to school. TR 168. Similarly, several courts have examined the nature and extent of necessary services to make a determination of whether the requested services were "medical" in nature. *See* § III.C, *supra*.

This balancing test is no more difficult to apply than other IDEA balancing tests. For example, the eligibility of a child for services is made only after an individualized consideration of testing and evaluation materials which must consider the nature and extent of the child's deviation from his or her nondisabled peers. 20 U.S.C. § 1412 (5)(C). Likewise, the courts apply balancing tests in determining the appropriate placement for a child and whether the "*nature and severity* of a disability is such that education in regular classes with the use of supplementary aids and services cannot be achieved satisfactorily." *Id.* § 1412(5)(B) (emphasis added). *A.W. v. Northwest R-1 Sch. Dist.*, 813 F.2d 158 (8th Cir.), *cert. denied*, 484 U.S. 847 (1987); *Schuldt v. Mankato Ind. Sch. Dist.*, 937 F.2d 1357 (8th Cir. 1991); *Roncker v. Walter*, 700 F.2d 1058 (6th Cir.), *cert. denied*, 464 U.S. 864 (1983); *Daniel R.R. v. State Bd. of Educ.*, 874 F.2d 1036 (5th Cir. 1989). Balancing tests are also used in making disciplinary decisions for children with disabilities. *Light v. Parkway C-2 Sch. Dist.*, 41 F.3d 1223 (8th Cir. 1994), *cert. denied*, 115 S.Ct. 2557 (1995). These determinations may lead to different decisions, including level of services, for similarly situated children across the country. But what is lost in consistency is gained in flexibility and practicality.

B. Respondent's Criticisms of the Balancing Test Are Misplaced.

Respondent argues that adoption of a bright-line physician/nonphysician distinction "eliminates the costs, delay, and the general state of confusion that would result from

the imposition of the multifactor test . . . which would require adjudication of varied factual circumstances through extensive appellate processes." However, the test proposed by the District is exactly the type of balancing test currently used in making eligibility, placement and service decisions under the IDEA. 20 U.S.C. § 1414(a)(5). The ease of applying the balancing test was demonstrated in the IEP team meeting for Garret in which determinations were made as to the services Garret needed, that Garret required continuous care by an appropriately licensed practitioner, that existing school nurses could not provide the services, and that his care, if not properly performed, could be life-threatening. EX 991 at 364-368; J.A. 14-16.

The bright-line physician/nonphysician test, while seemingly simple, in fact does not lead to uniformity or consistency, nor is it a viable test over time. "In recent years, nonphysician clinicians have provided increasing amounts of health care that, in the past, has been provided principally by physicians." Richard A. Cooper, *Roles of Nonphysician Clinicians as Autonomous Providers of Patient Care*, 280 JAMA 795 (1998). "Physicians are no longer seen as the sole provider" of "physician services." Richard A. Cooper, *Perspectives on the Physician Workforce to the Year 2020*, 274 JAMA 1534, 1541 (1995).

State and federal regulations that determine what functions nonphysicians are permitted to perform have seen sweeping changes in recent years. In 1995 alone, state legislatures enacted more than 300 laws regarding scope of practice and reimbursement for nonphysicians, "virtually all" of which "increase[d] the authority of nonphysicians in areas long reserved to physicians." Catherine M. Dower, *et al.*, *Changing Nature of Physician Licensure: Implications for Medical Education in California*, 168 Western J. of Med. 422, 424 (1998). In 1996, legislators introduced another 200 such bills and enacted 100 such laws. *Id. See also* Arlyss L. Anderson, *et al.*, *Practice Environment for Nurse Practitioners in California: Identifying Barriers*, 165 Western J. of Med. 209

(1996) (citing the "wide variation in the use and scope of practice of nurse practitioners from state to state").

Federal regulations also reflect the evolution of physicians' and nonphysicians' respective roles. OSHA, which traditionally had required physicians to perform evaluations mandated in health standards, recently began to allow nonphysician health professionals to perform evaluations. *See* Howard M. Sandler, *Move Over, Physicians: OSHA's New Approach to Evaluations,* 60 Occupational Hazards, April 1998, at 17. Insurers, including the federal government through Medicare, increasingly treat nonphysicians as covered "providers." Under the Balanced Budget Act, beginning this year nurse practitioners, clinical nurse specialists and physician assistants are treated as Medicare-covered providers.

C. The Government's Criticisms of the Balancing Test Are Misplaced.

The United States argues that the five-factor test proposed by the District is contradicted by examples of "related services" contained in the statute. Yet the United States fails to recognize that "psychological services" and "physical and occupational therapy," unlike medical services, are expressly required as related services. Thus, the continuous and extensive nature of those services is not a consideration in determining the student's entitlement to them. Congress may well have listed certain services as required in order that they would not be viewed as medical services; Congress did not list private-duty type nursing.

Similarly, the United States misreads the District's contention that consideration should be given to whether a *school* nurse can provide the services. The Department's own regulations define related services to include "school health services" which are "services provided by a qualified school nurse or other qualified person." 34 C.F.R. §§ 300.16(a) and 300.16(b)(11). In specifically requiring "school" health services and not just "health services," it was contemplated that some health services are not

required school health services, and in defining the services as provided by a "school" nurse, it must mean there was some recognition that school nurses are not private-duty nurses. One-on-one private duty-type nursing is not a service traditionally or generally provided by school nurses who provide services to several hundred students (TR 441-42, 470, 477-480), and certainly there is nothing in the record when the regulations were promulgated to indicate that the Secretary considered such intensive nursing services to be school health services. Therefore, school nurses may be required to provide certain intermittent health services required by children with disabilities, such as tube feedings and CIC (currently provided by District nurses) (TR 333-37, 380-83), since they can be provided in the context of their total school nursing duties. It is a far stretch to rule that districts must hire a nurse full-time for a single student.

D. The District Should Prevail Under a Balancing Test.

Respondent attempts to argue that he would prevail even under a balancing test. However, as has been previously pointed out, the ALJ in this case did not apply a true balancing test, but simply said that Garret was not as "fragile" as the students in some of the other lower court cases. Pet. App. 52a. When a true balancing is performed, all the relevant factors weigh in favor of classifying the services as "medical." Garret cannot be left without a trained attendant with him at all times to make nursing assessments. TR 44-45, 427, EX 91 at 388. The Iowa Board of Nursing has specifically ruled that a licensed nurse (minimally an LPN) is required to provide the services because of the nature and complexity of the care. The nurse may not see to the needs of any other children in the building or in any other buildings while attending to Garret. TR 469-470. Without the services provided by the LPN, Garret could be in a life-threatening situation at any time. Hiring a special LPN for Garret would cost over $30,000. Under a balancing test, the requested services are "medical."

VI. THIS CASE SHOULD NOT BE AFFIRMED ON STATE LAW GROUNDS.

Since neither of the lower federal courts addressed the state law issues, they should be considered on remand. Moreover, for all practical purposes, the relevant Iowa law is the same as the IDEA. If Petitioner prevails on the IDEA issues, it is entirely likely that the lower courts will find for Petitioner on the state-law issues.

The ALJ relied upon language in the Iowa statute which defines "special education" as including "corrective and supporting services." This language mirrors the definition of "related services" in the IDEA. *Compare* § 256B.2(3) of the Iowa Code *with* 20 U.S.C. § 1401 (a)(17). The Iowa Department of Education has also adopted the federal definition of "related services," including the specific exclusion of medical services. 281 Iowa Admin. Code § 41.94(2)(d) (1995). Furthermore, the Iowa Supreme Court has already ruled that variations in language between the federal and state laws do not grant greater rights of entitlement to special education services under state law. *Buchholtz v. Iowa Dept. of Public Instruc.*, 315 N.W. 2d 789 (Iowa 1982).

CONCLUSION

The decision of the Eighth Circuit Court of Appeals should be reversed.

Respectfully submitted,

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